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Sent electronically via regulations.gov

RE: Request for Information: Accelerating the Adoption and Use of Artificial Intelligence as Part of Clinical Care

Dear Dr. Keane,

On behalf of the American Urological Association (AUA), thank you for the opportunity to provide comments on the Office of the Deputy Secretary and Assistant Secretary for Technology Policy (ASTP) and Office of the National Coordinator for Health Information Technology (ONC)'s request for information (RFI) on Accelerating the Adoption and Use of Artificial Intelligence (AI) as Part of Clinical Care.

The AUA is a globally engaged organization with more than 22,000 members practicing in more than 100 countries. Our members represent the world's largest collection of expertise and insight into the treatment of urologic disease. Of the total AUA membership, more than 18,000 are based in the United States, therefore we are actively engaged in commenting on the regulatory proposals of the Department of Human and Health Services and its agencies. Our members provide invaluable support to the urologic community by fostering the highest standards of urologic care through education, research, and formulation of health policy.

With this background, we provide our comments on the use of AI in clinical care in response to the questions you posed in the RFI most relevant to our members.

2. What regulatory, payment policy, or programmatic design changes should HHS prioritize to incentivize the effective use of AI in clinical care and why? What HHS regulations, policies, or programs could be revisited to augment your ability to develop or use AI in clinical care? Please provide specific changes and applicable Code of Federal Regulations citations.

Predictable reimbursement is a primary driver of adoption of physician services and new technology and aligns incentives toward value rather than volume. The Department of Health and Human Services (HHS) should expand permanent payment mechanisms at the Centers for Medicare & Medicaid Services (CMS) for Food and Drug Administration (FDA)-cleared, high-value AI tools that demonstrate outcome improvement or workflow efficiency, while optimizing FDA oversight to ensure safety of AI in clinical care and to incentivize AI governance in hospital/patient safety programs.

To further incentivize the effective use of AI in clinical care, HHS should prioritize changes that align incentives with safe, validated, and clinically meaningful AI use rather than AI adoption alone. Rules that clearly define AI-enabled care and outline how payors, patients, and hospitals use these tools will be crucial in laying a solid foundation for future tools. AUA recommends that HHS consider the key priorities listed below:

- **Payment alignment**: CMS should create explicit payment pathways (e.g., HCPCS G-codes or add-ons) for AI-assisted data review, risk stratification, triage, documentation support, care planning, and clinical decision review when integrated into certified EHR workflows and reviewed by a clinician. Clinician time spent on required post-deployment monitoring of AI for data drift, bias, and safety should also be recognized. (Applicable CFR: 42 CFR Part 414)
- **Cost recognition via inclusion in value-based programs**: In the Merit-based Incentive Payment System (MIPS) and advanced Alternative Payment Models (APMs), AI licensing, validation, updates, and monitoring should be treated as allowable practice expense inputs when tied to clinical services to avoid underinvestment in governance and safety. (Applicable CFR: 42 CFR 414.1350) HHS should also consider incentivizing AI generation and its use by making an improvement activity category for "responsible AI deployment" that requires model documentation, workflow integration, drift monitoring, subgroup performance checks, and clinician override logging. (Applicable CFR: 42 CFR 414.1355)
- **Inpatient adoption**: CMS should clarify that software-based AI, including non-medical device tools, may qualify for New Technology Add-On Payments in the inpatient setting when criteria are met and provide AI-specific evidentiary guidance. (Applicable CFR: 42 CFR 412.87)
- **Electronic Health Records (EHR) guardrails**: HHS should preserve and enforce ONC predictive decision support requirements for transparency, risk analysis, and mitigation at the point of care. (Applicable CFR: 45 CFR 170.315(b)(11))
- **Interoperability and oversight**: Information-blocking exceptions should be applied narrowly so they do not impede lawful access for AI evaluation and monitoring. (Applicable CFR: 45 CFR Part 171)
- **Privacy clarity**: HHS should issue HIPAA guidance clarifying de-identification and minimum-necessary standards for AI development, validation, and post-deployment monitoring. (Applicable CFR: 45 CFR 164.514)

3. For non-medical devices, we understand that use of AI in clinical care may raise novel legal and implementation issues that challenge existing governance and accountability structures (e.g., relating to liability, indemnification, privacy, and security). What novel legal and implementation issues exist and what role, if any, should HHS play to help address them?

Clinicians recognize AI's potential to improve care delivery but are concerned about unresolved safety, legal, and operational risks. When AI contributes to patient harm, accountability is often blurred among clinicians, health systems, vendors, and EHR platforms. In addition, contracts may shift indemnification risk to clinicians despite their limited control over AI design or updates.

Clinicians are also wary of silent updates and model drift that can alter performance without notice, as well as documentation risks from generative AI that may produce credible but inaccurate content, if not carefully reviewed and labeled. Other concerns include privacy and cybersecurity vulnerabilities, such as prompt injection, PHI leakage, secondary use of data, and risks from third-party integrations, along with limited mechanisms to monitor and address bias across patient populations. To address these concerns, clinicians could benefit from model contracting guidance issued by HHS, stronger documentation transparency standards, and the creation of a national learning system to monitor and improve AI-related safety outcomes.

5. How can HHS best support private sector activities (e.g., accreditation, certification, industry-driven testing, and credentialing) to promote innovative and effective AI use in clinical care?

Proper validation of AI platforms is of utmost importance to physicians and patients. With any new technology, a thorough and transparent process must be in place prior to approval. **We recommend that HHS streamline the process necessary for an AI platform to gain accreditation or approval by aligning the market around common, measurable expectations (e.g., a set of minimum assurance elements including governance, clinical validation, cybersecurity, etc.) while avoiding duplicative federal bureaucracy.** When developing the accreditation process, we ask that HHS consider specialty society participation in defining clinically meaningful endpoints and minimum evidence thresholds so that accreditation reflects real workflow and outcome needs, not just technical performance.

HHS should support public–private partnerships that enable independent, third-party testing of AI models across populations, sites, and time, including post-deployment monitoring. A nationwide network of AI assurance laboratories, supported but not operated by HHS, would allow industry-driven testing results to be shared transparently and reused across health systems. This would help avoid duplicative work across health systems that will slow innovation and create barriers in reaching our most vulnerable patients. HHS should consider these strategies to support this third-party testing:

1) Create a federal recognition pathway that identifies credible third-party programs aligned with transparency, risk management, and post-deployment monitoring expectations, and that require public-facing disclosures of intended use, limitations, and update governance.
2) Provide grants for demonstration projects. HHS could support a testing infrastructure that allows vendors and health systems to compare performance, subgroup equity, calibration, and drift across settings, and to publish standardized “model facts” for clinical users. HHS could provide access to large databases that are supported by the government like the Veterans Health Administration’s system for AI learning and help solve HIPPA hurdles as they arise.
3) Create a "virtual healthcare system" of sample data (images, labs, notes, etc.) upon which private sector AI tools could be tested/validated. This resource would be helpful for safety purposes and comparative effectiveness and would incentivize independent testing for tools that demonstrate third-party evaluation, real-world monitoring plans, and interoperability conformance.

4) Extend ONC certification so that there is a higher internal standard for AI-workflows within health systems

6. Where have AI tools deployed in clinical care met or exceeded performance and cost expectations and where have they fallen short? What kinds of novel AI tools would have the greatest potential to improve health care outcomes, give new insights on quality, and help reduce costs?

AI tools have met or exceeded expectations when narrowly scoped, integrated into workflow, and evaluated in real-world settings. For instance, AI tools targeting administrative burden and narrow tasks perform well. AI scribes provide concise summaries of information for help with clinical decision-making and reduce documentation time and burnout without compromising quality. In urology, AI used for imaging triage, prostate cancer risk stratification, and operational workflows has shown value when tightly integrated into care delivery. Applications for workflows include handling patient communications with the front office, simple clinical communications with patients, scheduling, authorizations, and charting.

However, clinical AI often underperforms due to poor EHR integration, alert fatigue, and lack of reimbursement. AI scribes improve efficiency and documentation quality but have no sustainable reimbursement pathway, limiting adoption. AI has fallen short in reliably excluding errors when deployed as black boxes, are poorly calibrated, or are misaligned with clinical operations. Some AI tools often increase alert burden or create “extra clicks”; introduce documentation errors without clear labeling and review; or are based on models that degrade over time due to drift or population shift. AI tools in digital pathology, even though increasingly powerful and accepted within clinical use, have not exceeded cost-saving expectations.

In urology, AI that synthesizes the longitudinal chart data from electronic health records (EHR)—highlighting rising Prostate-specific antigen (PSA) trends, missed hematuria workups, medication risks, or post-op complications—could significantly improve patient safety. Additionally, next-generation clinical decision support tools should prioritize risk rather than add alerts (e.g., identifying patients at highest risk for urinary retention, infection, or delayed cancer diagnosis). AI tools that reach the bedside and nursing teams, such as early warning for catheter-associated complications or post-procedure deterioration, offer significant potential to improve outcomes while reducing costs.

Lastly, future impactful tools may focus on creating a primary care infrastructure that helps reduce the number of routine visits with care providers and engages patients in their health care decisions. AI tools to improve information sharing, even without consolidation of services, would decrease fragmentation of care and may preserve coordinated care while still fostering competitive efforts.

7. Which role(s), decision maker(s), or governing bodies within health care organizations have the most influence on the adoption of AI for clinical care? What are the primary administrative hurdles to the adoption of AI in clinical care?

This topic merits additional exploration, as our experts surveyed provided a wide range of perspectives and no clear consensus. We also note that every health care organization seems to have its own process to make decisions regarding AI. have designated committees while others do not, and the makeup of such committees can heavily influence the likelihood of AI adoption.

Our members reported that the following individuals within their health systems have the most influence: Chief Medical Information Officer (CMIO)/clinical informatics leadership, the Chief Information Officer (CIO) and Chief Information Security Officer (CISO), Chief Quality and Safety Officer, compliance and privacy officers, legal and contracting, value analysis or digital governance committees, service line leadership and department chairs. Several members mentioned that professional medical specialty societies, especially trade associations, often carry the most influence on adopting medical technologies and service lines. For instance, large group practices, institutions such as Kaiser, and academic programs demonstrate the clinical and financial efficacy of a new technology, and wider adoption eventually follows. The key players in these organizations include thought leaders in clinical practice and/or chief scientific officers, administrative managers of clinical and business operations (CEO/CFO), and nurse managers.

One member said that federal government bodies such as the department of HHS or CMS would play the most important role. For instance, CMS might play a disproportionately important role in adoption for clinical care as they would design and implement reimbursement incentives. Similarly, HHS may be able to mandate documentation of decisions made with the assistance of AI tools, allowing for retrospective and comparative analyses of decisions generated by the presence/absence of AI assistance.

There are multiple administrative hurdles to adopting AI in clinical care. These include vendor risk and security assessments, contracting and indemnification (audit rights, update control, data retention), EHR integration capacity and change control, workflow redesign and training, lack of internal evaluation and monitoring infrastructure, unclear return of investment due to misaligned incentives where costs sit with clinics, but savings accrue to payers or the broader system.

8. Where would enhanced interoperability widen market opportunities, fuel research, and accelerate the development of AI for clinical care? Please consider specific data types, data standards, and benchmarking tools.

AI platforms should be required to be interoperable. Enhancing interoperability specifically in the areas listed below would not only improve access to longitudinal, multi-source data, but also enable consistent post-deployment monitoring and benchmarking.

- **High-value data types**: Longitudinal labs, vitals, meds, problems, allergies; imaging and structured reports; operative notes and pathology; wearable device data; patient-reported outcomes; claims and encounter data; referrals and care transitions; scheduling and utilization signals.
- **Data standards and implementation needs**: Scalable Fast Healthcare Interoperability Resources (FHIR) read and write capabilities, provenance, and event notifications; ONC certification for deeper in-system integration and higher security standards to support

and enhance trust; the Trusted Exchange Framework and Common Agreement (TEFCA) interoperability for patient care across state lines; better Digital Imaging and Communications in Medicine (DICOM) interoperability; standardized representation of AI outputs in the record (tool name/version, output, confidence or risk category, clinician action/override); and consistent patient identity matching and metadata.

- **Benchmarking tools**: federated evaluation frameworks that allow models to be tested across sites without exporting raw protected health information (PHI); standardized test harnesses and reporting templates for calibration, subgroup equity, drift, and clinical utility; and shared registries for post-market performance learning.

9. What challenges within healthcare do patients and caregivers wish to see addressed by the adoption and use of AI in clinical care? Equally, what concerns do patients and caregivers have related to the adoption and use of AI in clinical care?

Patients express interest in AI as a tool to improve access to health care expertise, particularly if it offers timely, convenient, and confidential support without replacing human judgment, interaction, and compassion. However, it must come at a reasonable cost. They hope AI can streamline scheduling and triage, enhance education and care navigation, improve coordination among providers, reduce administrative delays, strengthen follow-up and symptom monitoring, minimize redundant testing, and support more personalized care planning. Many value AI that helps clinicians identify important information in large medical records and as tools that support daily care while preserving patient autonomy.

However, they have significant concerns about privacy, data security, and reliability. Specifically, they worry about secondary uses of their data, unclear data retention practices, lack of transparency about when AI is used, erosion of clinician accountability, and the use of recording or ambient documentation without clear patient consent.

Addressing these concerns requires building trust through transparency about when and how AI is used, maintaining clinician oversight, offering opt-out options for nonessential uses, ensuring strong privacy and security protections, and supporting ongoing monitoring with public accountability for safety and equity. Patients generally support AI when it enhances clinical care while respecting consent, privacy, and dignity; failure to meet these expectations may undermine trust, even in effective tools.

10. Are there specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care?

Current evidence on AI's impact in health care is mixed, and there is an urgent need for real-world evaluation. For instance, AI documentation tools have reduced administrative burden in some settings, but effects on burnout, care quality, and costs remain inconsistent. Clinical prediction models may also perform poorly in practice, and while economic studies often report AI as cost-effective or cost-saving, results vary by clinical application and methodology, and negative findings may be underreported.

Also, cost-effectiveness studies often fail to clarify who bears costs versus who gains benefits when using AI and frequently overlook full lifecycle expenses, including integration, cybersecurity, monitoring, revalidation, and governance. They also rarely address model updates over time or how financial impacts are distributed across stakeholders, and study quality varies. HHS could help address these gaps by supporting standardized evaluation frameworks that capture total cost of ownership, clearly map cost and benefit distribution, and require real-world monitoring to ensure sustained value after deployment.

Below are several areas that need further research and should be prioritized:

- **Real-world clinical utility and safety**: Pragmatic trials and stepped-wedge evaluations of AI tools embedded in routine workflows, with standardized outcome measures (harms, time-to-intervention, complications, downstream utilization) rather than accuracy alone. There should also be safety studies to determine where and when to trust AI and what standards are best to ensure transparency.
- **Health economics and implementation science**: Standard methods to measure total cost of ownership (integration, security, monitoring, clinician oversight), budget impact, and distribution of costs and savings across stakeholders, plus implementation playbooks for small/rural and safety-net settings.
- **Post-deployment monitoring science**: Methods and infrastructure for drift detection, recalibration, subgroup surveillance, and "override analytics" (when clinicians disagree with AI and why), including standardized incident reporting taxonomies for AI-related safety events.
- **Human factors and human-AI teaming**: Usability, cognitive load, automation bias, clinician trust calibration, and patient communication studies that determine where AI improves decisions versus adds noise.
- **Generalizability and robustness**: Multisite external validation, dataset shift testing, and robustness benchmarking across institutions, EHR configurations, documentation styles, and patient subgroups.
- **Interoperability-enabled evaluation**: Federated evaluation toolkits and shared test harnesses that enable benchmarking without moving raw PHI (and that include provenance and model versioning).

Thank you again for the opportunity to provide comments on this important issue. Should you have any questions, please contact Melika Zand, Patient Advocacy Manager, at mzand@auanet.org.

Sincerely,